

Tue, Jun 30 at 10:08 PM The

ankle dorsiflexion checked by the physiotherapist is 0/5, and the one checked by the neurologist is 3/5.

In such cases, when writing the NDIS High Cost AT Report or Bioness L300 Go Report, it is best to explain the difference between the two figures as they are, without hiding the fact that they are different.

Manual Muscle Testing (MMT) can vary depending on the examiner, examination posture, allowance of compensatory movements, fatigue level, and the timing of the test. Variability is particularly common in patients with MS.

The report can be written as follows.

Muscle Strength Assessment

There is variability in the documented right ankle dorsiflexion strength across assessments.

Neurologist assessment: Right ankle dorsiflexion 3/5.

Physiotherapy assessment: Right ankle dorsiflexion 0/5.

The difference between assessments is likely related to variations in testing conditions, fatigue levels, examiner technique, and the fluctuating nature of Multiple Sclerosis. During orthotic assessment and gait observation, the participant demonstrated significant functional foot drop with inadequate active dorsiflexion during swing phase, resulting in reduced toe clearance and increased risk of tripping.

Regardless of the numerical MMT grading recorded by different clinicians, all assessments consistently identified clinically significant dorsiflexion weakness that adversely affects gait safety, efficiency, and community mobility.

Additionally, the Bioness report actually considers functional evidence to be more important than MMT figures.

For example, the persuasiveness becomes much stronger if the following content is included.

Foot drop observed during gait

Reduced toe clearance

Frequent tripping episodes

Compensatory hip hiking/circumduction

Increased falls risk

Improvement during Bioness trial

Decline during washout period

If this is the final submission, I recommend not assuming Baseline Strength is 0/5, but rather summarizing it as "0–3/5 documented across clinical assessments," and connecting the key point to "functional foot drop was consistently present."

NDIS examiners place more importance on the actual decline in walking function and the effects of AT use than on the muscle strength numbers themselves.